Psychological Disorders and Therapies

What counts as a disorder, how common disorders are in India and worldwide, the main categories, which treatments work — and what a manager should and shouldn't do when someone on the team is struggling.

Isagi · November 2026 · Spine: Ciccarelli & White, *Psychology* (4th ed.), Ch. 14–15 — rewritten, updated and applied, at overview level

In any team of forty people, several are likely to be living with a mental-health condition this year — most often anxiety or depression — and most will not have told anyone. Managers aren't clinicians and shouldn't try to be. But knowing the basics helps in three ways: noticing when something is wrong, responding without making it worse, and knowing that effective treatments exist. This final part is an overview of two textbook chapters, and it deliberately stays at that level.

1 What makes behaviour a disorder

No single test separates "disordered" from "normal". Psychologists combine several criteria:

- **Statistical rarity** — unusual behaviour. But rarity alone isn't illness: exceptional talent is rare too.
- **Violating social norms** — but norms differ across cultures and change over time.
- **Personal distress** — the person suffers. Important, but some conditions don't cause distress to the person who has them.
- **Impaired functioning** — difficulty working, maintaining relationships or caring for oneself.

The working definition used today: a **psychological disorder** is a pattern of thoughts, feelings or behaviour that causes significant distress or impairs a person's ability to function, and is not simply an expected reaction to a situation (grief after a death, for example).

2 What causes disorders

Several models offer explanations. The **biological** model looks at genes, brain chemistry and brain structure. **Psychological** models look at learned behaviour, patterns of thinking and early experiences. The **sociocultural** model looks at poverty, discrimination, family and culture. Most clinicians now use the **biopsychosocial** model, which combines all three, and a **vulnerability–stress** (diathesis–stress) view: a person may carry a vulnerability — genetic or from early life — that becomes a disorder only when enough stress is added. That is why chronic workplace stress (Part 9) matters: it can be the added load that tips a vulnerable person into illness.

3 How disorders are classified

Two systems are used worldwide. The American Psychiatric Association's **DSM-5-TR** (2022) describes each disorder by its symptoms, duration and effects. The World Health Organization's **ICD-11** (in effect from 2022) is the international standard, and the system used in India's public health records. Diagnosis enables treatment, research and insurance. It also carries a cost: labels can stick and stigmatise. Since 2019 ICD-11 has also listed burnout — as an occupational phenomenon, not a mental disorder (Part 9).

4 How common they are

The WHO's 2022 World Mental Health Report estimated that in 2019 about **970 million people** — roughly **1 in 8** worldwide — were living with a mental disorder, most commonly anxiety and depression. In India, the National Mental Health Survey of 2015–16 estimated that about **1 in**

10 adults had a current mental disorder, and that the large majority of people who needed treatment did not receive it — a treatment gap the survey put at roughly 70–90% depending on the disorder.

WORKED EXAMPLE 1 · WHAT PREVALENCE MEANS FOR ONE TEAM

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Team size:                      40 people
At about 1 in 10 (India):        40 × 0.10 = 4
At about 1 in 8 (global):        40 × 0.125 = 5

Treatment gap of ~80%:           of 4, about 3 untreated
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On average, four or five people in a team of forty, of whom perhaps three are not getting help. These are averages and the actual number varies, but the point holds: mental-health conditions in a team are normal, not exceptional, and most are invisible. The aim is not to identify them, but to make it easy for people to seek help and to adjust work while they do.

5 The main categories

Category	Core features	Notes
Anxiety disorders	Excessive fear or worry: specific phobias, social anxiety, panic attacks, generalised anxiety	The most common group. Highly treatable.
Obsessive–compulsive disorder	Intrusive, unwanted thoughts (obsessions) and repetitive acts to reduce the anxiety (compulsions)	Not the same as liking things tidy.
Trauma-related (e.g. PTSD)	Re-living a traumatic event, avoidance, feeling constantly on guard	Can follow accidents, assault, disasters.
Depressive disorders	Persistent low mood or loss of interest, with changes in sleep, energy, appetite and concentration, for at least two weeks	Among the leading causes of disability worldwide.
Bipolar disorder	Periods of depression alternating with mania: elevated mood, little need for sleep, racing ideas, risky decisions	Usually needs long-term medical treatment.
Eating disorders	Serious disturbances in eating and in thoughts about food, weight and body	Can be life-threatening; specialist care needed.
Schizophrenia	**Positive** symptoms (hallucinations, delusions, disorganised speech) and **negative** symptoms (flat emotion, withdrawal)	Affects fewer than 1 in 100 people. Not "split personality".
Personality disorders	Long-standing, inflexible patterns that cause problems across situations, e.g. borderline or antisocial personality disorder	Distinct from ordinary personality differences (Part 11).
Dissociative disorders	Disruptions in memory, identity or awareness	Dissociative identity disorder remains debated.

6 Psychological therapies

Therapies divide broadly into **insight therapies**, which aim at understanding, and **action therapies**, which aim at changing behaviour and thinking directly.

- **Psychodynamic therapy** — Freud's psychoanalysis explored the unconscious through free association and dream analysis, over years. Modern psychodynamic therapy is shorter and more focused on current relationships.
- **Person-centred therapy** (Carl Rogers) — the therapist offers **empathy**, **genuineness** and **unconditional positive regard**, and the client leads. Its core skills — reflective listening, not judging — are useful well beyond therapy.
- **Behaviour therapy** applies learning principles (Part 4). **Exposure therapy** confronts feared situations gradually and safely until the fear fades; **systematic desensitisation** pairs gradual exposure with relaxation. Exposure is among the most effective treatments for phobias, OCD and PTSD.
- **Cognitive therapy** (Aaron Beck) and **rational emotive behaviour therapy** (Albert Ellis) target distorted thinking. Combined with behavioural methods, this is **cognitive behavioural therapy (CBT)**, the most widely tested talking therapy, effective for depression, anxiety, insomnia and more.
- **Group** and **family therapy** treat problems in the context of relationships. Family involvement is particularly relevant in India, where care is often family-centred.

Cognitive distortions

Beck identified recurring patterns of distorted thinking. They are worth knowing because everyone uses them under stress, and they show up constantly in sales reviews:

Distortion	Example
All-or-nothing	"I missed target, so I'm a failure."
Overgeneralisation	"I lost this dealer. I always lose the good ones."
Catastrophising	"One bad quarter and I'll be fired."
Mind reading	"The RSM thinks I'm useless."
Personalisation	"The region missed because of me."

WORKED EXAMPLE 2 · A THOUGHT RECORD

CBT's basic tool is a **thought record**: write the situation, the automatic thought, the evidence for and against, and a more balanced thought. A rep after a poor quarter:

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Situation:   Q2 at 78% of target
Thought:     "I'm going to be fired."   (catastrophising)
Evidence for: the review is next week
Evidence against: Q1 was 104%; region at 85%;
             nobody fired for one quarter in 3 years
Balanced:    "Q2 was weak, partly the region.
             I need a plan for Q3 — not a resignation."
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The thought changes from a catastrophe to a problem with a next step. This is a self-help technique for everyday stress, not a substitute for treatment — but managers who coach this way, asking "What's the evidence?", help people think more clearly under pressure.

Does therapy work?

Yes. A 1977 meta-analysis by Mary Lee Smith and Gene Glass, and a larger follow-up in 1980, found that the average person in therapy was better off than about 75–80% of comparable untreated people. Many therapies perform similarly for many problems, which points to **common factors** — above all the **therapeutic alliance**, the working relationship between client and therapist. Some treatments are clearly better for specific problems: exposure for anxiety disorders, CBT for depression and insomnia. In India, where trained therapists are scarce, trials have

shown that lay counsellors trained in simple structured therapies can deliver real benefits — one 2017 trial in Goa found a brief behavioural therapy delivered by lay counsellors reduced depression.

7 Biomedical treatments

Medications are prescribed by doctors, most often psychiatrists. **Antipsychotics** reduce the positive symptoms of schizophrenia. **Antidepressants**, most commonly SSRIs (Part 1), are used for depression and anxiety. **Anti-anxiety** medicines such as benzodiazepines act quickly but carry a risk of dependence, so are generally used short-term. **Mood stabilisers** such as lithium are used for bipolar disorder. A 2018 analysis of over 500 trials found all 21 antidepressants studied were more effective than placebo for acute depression in adults, though the average advantage was modest. Medication and CBT combined often work better than either alone. **Electroconvulsive therapy (ECT)**, now given under anaesthesia, remains an effective treatment for severe depression that hasn't responded to other treatments — far removed from its film image.

8 What a manager should do

India's **Mental Healthcare Act, 2017** gives every person a right to access mental-health care and protection from discrimination, and it effectively decriminalised attempted suicide by presuming that a person who attempts it is under severe stress. The legal backdrop has moved toward treatment and away from blame. A manager's role is narrow and important:

NOTICE, ASK, LISTEN, POINT, ADJUST

1. **Notice** changes that last for weeks: withdrawal, irritability, missed deadlines in someone usually reliable, visible exhaustion.
2. **Ask** privately and simply: "You haven't seemed yourself lately. How are you doing?"
3. **Listen** without diagnosing, fixing or comparing. Take it seriously.
4. **Point** to help: the company's employee assistance programme if there is one, a doctor, or **Tele-MANAS** (dial **14416**), India's free, round-the-clock mental-health helpline. In an emergency, call **112**.
5. **Adjust** work while they recover — flexible hours, lighter targets, fewer long drives — and keep what you've been told confidential.

If someone talks about not wanting to live, or about suicide, take it seriously, stay with them or make sure they are not alone, and connect them to help immediately — Tele-MANAS or emergency services.

9 Held up?

Claim	Verdict	Why
Psychotherapy works	**Yes**	Consistent across decades of meta-analyses; the alliance matters most.
Depression is simply a "chemical imbalance"	**Oversimplified**	A 2022 review found no consistent evidence for low serotonin as the cause. Antidepressants still help many people, by mechanisms not fully understood.
Rosenhan's 1973 "sane in insane places" study proved psychiatric diagnosis is meaningless	**Doubtful**	Later investigation of the study's records raised serious questions about how it was carried out and reported.
Venting anger (punching a pillow) gets it out of your system	**No**	Experiments find venting tends to increase later aggression.
ECT is outdated and harmful	**No**	Under modern anaesthesia it is an effective treatment for severe, treatment-resistant depression.
Lay counsellors can deliver effective therapy	**Yes, for some conditions**	Supported by trials in India and other low-resource settings.

Summary

- A **psychological disorder** is a pattern of thought, feeling or behaviour causing significant distress or impairment, beyond an expected reaction to events.
- Causes are **biopsychosocial**: vulnerability plus stress. Chronic work stress can be the added load.
- Disorders are classified by **DSM-5-TR** and **ICD-11**. Labels help treatment but can stigmatise.
- About 1 in 8 people worldwide live with a mental disorder; in India about 1 in 10 adults, with most untreated. In a team of 40, expect four or five.
- Main groups: anxiety, OCD, trauma-related, depressive, bipolar, eating, schizophrenia, personality and dissociative disorders.
- Therapies: psychodynamic, person-centred, behaviour therapy (exposure), cognitive therapy and **CBT**. Therapy works — the average client ends up better off than about 75–80% of untreated people — and the alliance matters most.
- Medications (antipsychotics, antidepressants, anti-anxiety drugs, mood stabilisers) and ECT help specific conditions, often best combined with therapy.
- Managers: notice, ask, listen, point to help (Tele-MANAS 14416; emergencies 112), adjust work — and don't diagnose.

Practice problems

P1. A region has 250 employees. Using India's estimate of about 1 in 10 adults with a current disorder and a treatment gap of about 80%, estimate how many may be affected and how many untreated.

P2. Name the cognitive distortion: (a) "If this launch isn't perfect, the whole year is ruined"; (b) "My manager didn't reply — he must be angry with me"; (c) "One dealer complained, so all dealers hate our service."

P3. Write a thought record for a rep whose automatic thought is "I'll never be good at key accounts" after losing one pitch.

P4. A usually reliable team member has seemed withdrawn and exhausted for a month and has missed two deadlines. Script your first conversation in three or four lines.

P5. A colleague says "Therapy is just talking; it doesn't actually do anything." What does the evidence say?

P6. List three work adjustments you could offer a field rep recovering from depression, without disclosing anything to the team.

Glossary

Psychological disorder

A pattern causing significant distress or impaired functioning.

Biopsychosocial model

Disorders arise from biological, psychological and social factors combined.

Vulnerability–stress model

A predisposition becomes a disorder when enough stress is added.

DSM-5-TR / ICD-11

The US psychiatric manual / the WHO's international classification.

Treatment gap

The share of people needing care who don't receive it.

Positive / negative symptoms

Added experiences such as hallucinations / losses such as flat emotion.

Insight / action therapy

Therapy aimed at understanding / at direct change.

Person-centred therapy

Rogers's client-led therapy built on empathy and acceptance.

Exposure therapy

Gradual, safe confrontation of feared situations.

CBT

Cognitive behavioural therapy: changing distorted thoughts and behaviours.

Cognitive distortion

A habitual error in thinking, such as catastrophising.

Thought record

A CBT worksheet testing an automatic thought against evidence.

Therapeutic alliance

The working relationship between client and therapist.

SSRI

A common class of antidepressant acting on serotonin.

ECT

Electroconvulsive therapy, used under anaesthesia for severe depression.

Answer key

P1. 250 × 0.10 = about 25 people; 25 × 0.80 = about 20 untreated. Averages only.

P2. (a) all-or-nothing; (b) mind reading; (c) overgeneralisation.

P3. Situation: lost the builder pitch. Thought: "I'll never be good at key accounts" (overgeneralisation). Evidence for: lost this one. Evidence against: won two smaller institutional deals this year; the winner offered 90-day credit we couldn't match. Balanced: "I lost on credit terms, not skill. I'll ask for feedback and prepare the next pitch differently."

P4. For example: "I've noticed you've seemed really tired the last few weeks, and that's not like you. How are you doing? … There's no pressure to tell me anything, but if something's going on, we can look at your workload, and the EAP or Tele-MANAS (14416) are there too."

P5. Meta-analyses since 1977 show the average person in therapy ends up better off than about 75–80% of comparable untreated people, with specific therapies such as CBT and exposure especially effective for depression and anxiety.

P6. For example: flexible start times for appointments; a temporarily reduced target or territory; fewer overnight trips and long drives; regular, private check-ins.

Help: Tele-MANAS, India's free 24×7 mental-health helpline: **14416**. Emergency: **112**.

Sources: chapter structure follows Ciccarelli & White, *Psychology*, 4th ed. (Pearson), Ch. 14–15; text is original. WHO, *World Mental Health Report*, 2022. National Mental Health Survey of India, 2015–16 (NIMHANS). Smith & Glass, *American Psychologist*, 1977. Patel et al., *The Lancet*, 2017. Cipriani et al., *The Lancet*, 2018. Serotonin review: Moncrieff et al., *Molecular Psychiatry*, 2022. Venting: Bushman, *Personality and Social Psychology Bulletin*, 2002. Mental Healthcare Act, 2017, s. 115. Worked-example figures are illustrative averages.